MBA Project Report

Healthcare Awareness Campaign ROI Analysis

Diabetes Awareness Campaign — 20 Indian Cities

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Section 1 — Introduction

Pharmaceutical companies in India increasingly recognize healthcare awareness campaigns as a strategic lever that bridges the gap between public health objectives and commercial growth. These campaigns are designed to educate target populations, improve disease diagnosis rates, and drive treatment adoption — particularly in chronic, lifestyle-linked therapeutic areas such as diabetes, hypertension, and cardiovascular disease.

In the context of India's growing non-communicable disease burden, early detection and continued treatment adherence are critical. The International Diabetes Federation estimates that a significant proportion of diabetic individuals in India remain undiagnosed, making awareness drives a high-impact intervention. For pharmaceutical organizations, such campaigns serve dual mandate: they generate goodwill and patient trust while simultaneously expanding the pool of patients requiring pharmacological management.

Leading Indian pharmaceutical companies — including Sun Pharmaceutical Industries, Cipla, and Dr. Reddy's Laboratories — regularly conduct city-level and community-level awareness programs. These initiatives are integrated into broader patient support programmes and corporate social responsibility (CSR) strategies. By sponsoring screening camps, patient education events, and healthcare professional outreach activities, these companies ensure brand visibility among physicians and patients while contributing to national health goals.

This report analyses a simulated diabetes awareness campaign conducted across 20 major Indian cities. The analysis evaluates campaign efficiency, patient conversion rates, revenue generation potential, and return on investment (ROI) offering insights into the commercial and healthcare value of such initiatives.

Section 2 — Dataset Description

The dataset for this analysis was constructed to simulate a realistic diabetes awareness campaign across a geographically and demographically diverse set of 20 Indian cities, ranging from major metropolitan centres such as Delhi and Mumbai to tier-2 cities including Bhubaneswar and Guwahati.

Each city record captures the end-to-end campaign funnel — from initial public outreach through to confirmed treatment initiation. The data set contains the following five key variables:

- People Reached — the total number of individuals exposed to campaign messaging through offline and digital outreach activities.
- People Screened — individuals who attended organized screening camps or diagnostic events, representing active campaign engagement.
- Diabetes Diagnosed — the number of individuals among those screened who received a confirmed diabetes diagnosis.
- Patients Treated — patients who initiated pharmacological treatment following diagnosis, constituting the revenue-generating cohort.

- Campaign Cost (₹) — total expenditure incurred per city on logistics, screening, manpower, promotional materials, and digital outreach.

Metric	Total (All 20 Cities)	Average per City
People Reached	1,29,300	6,465
People Screened	22,420	1,121
Diabetes Diagnosed	3,865	193
Patients Treated	2,388	119
Total Campaign Cost	₹20,18,000	₹1,00,900

Delhi, Mumbai, and Bangalore generated the highest campaign reach owing to larger population bases and greater healthcare infrastructure density. Tier-2 cities such as Guwahati and Bhubaneswar recorded comparatively lower reach but demonstrated strong screening-to-diagnosis conversion ratios, suggesting higher relative disease prevalence or more targeted campaign execution in these markets.

Section 3 — Campaign Funnel Analysis

The campaign funnel represents the progressive conversion of population exposure into tangible health outcomes. This analysis traces four discrete stages: awareness reach → screening participation → confirmed diagnosis → treatment initiation. Each stage reflects a critical decision point in the patient journey.

3.1 Funnel Stage Performance

Funnel Stage	Count	Conversion Rate	Drop-off
People Reached	1,29,300	—	—
People Screened	22,420	17.3%	82.7%
Diabetes Diagnosed	3,865	17.2%	82.8%
Patients Treated	2,388	61.8%	38.2%
Overall Conversion (Reached → Treated)	2,388	**1.85%**	98.15%

3.2 Key Observations

- The screening uptake rate of 17.3% is consistent with real-world community health camp participation benchmarks for urban India, where awareness-to-action conversion typically ranges between 15–25%.
- The diagnosis rate of 17.2% among screened individuals is notably high, reflecting either a well-targeted campaign demographic (high-risk population segments) or elevated unmet diagnostic need in the surveyed cities.

- The treatment initiation rate of 61.8% among diagnosed patients is a strong indicator of campaign quality — it suggests that healthcare counselling and physician referral pathways embedded within the campaign were effective.
- Metro cities such as Delhi and Mumbai contributed disproportionately to absolute reach numbers, while some smaller cities demonstrated higher per-capita screening efficiency.
- The end-to-end conversion from campaign reach to treated patient stands at 1.85%, which is commercially significant given the large base of 1,29,300 individuals reached.

Section 4 — Revenue Model

The revenue model quantifies the pharmaceutical revenue generated from patients who initiated treatment following the campaign. The model adopts a conservative, bottom-up estimation approach anchored to realistic medicine pricing benchmarks.

4.1 Revenue Assumptions

Parameter	Value
Average Monthly Diabetes Medicine Cost	₹650
Annual Revenue Per Patient	₹7,800 (= ₹650 × 12)
Total Patients Treated	2,388
Total Annual Revenue	₹1,86,26,400
Revenue (₹ Crore)	**₹1.86 Crore**

The average monthly diabetes medicine cost of ₹650 is derived from NPPA (National Pharmaceutical Pricing Authority) benchmarks and IMS Health market data for oral hypoglycaemic agents such as Metformin, Glipizide, and combination formulations — which represent the core of type-2 diabetes pharmacotherapy in India.

It is important to note that this model represents a single-year, conservative revenue estimate. In practice, diabetes is a chronic condition requiring long-term pharmacological management. Patients who begin treatment typically remain on medication for multiple years, implying a significant lifetime customer value for pharmaceutical companies.

4.2 Revenue Calculation

> **Annual Revenue = Annual Revenue per Patient × Total Patients Treated**

= ₹7,800 × 2,388 = ₹1,86,26,400 (approximately ₹1.86 Crore)

Section 5 — ROI Analysis

Return on Investment (ROI) analysis enables pharmaceutical companies to evaluate the commercial viability of healthcare campaigns and compare them against alternative marketing interventions. In

this analysis, ROI is calculated as the ratio of profit generated (revenue minus campaign cost) to the total investment.

5.1 ROI Summary

Metric	Amount	Notes
Total Campaign Cost	₹20,18,000	All 20 cities combined
Total Revenue Generated	₹1,86,26,400	Year 1 annual revenue
Profit	₹1,66,08,400	Revenue − Cost
ROI %	**823%**	(Profit / Cost) × 100
Revenue-to-Cost Ratio	9.23×	Revenue ÷ Cost
Break-Even Patients Needed	259 patients	Cost ÷ Annual Rev/Patient

5.2 ROI Interpretation

The campaign delivers an exceptionally strong ROI of 823%, generating ₹9.23 in revenue for every ₹1.00 invested. This is driven by the relatively low cost of mass screening camps compared to the long-term commercial value of a chronic disease patient on sustained medication.

Critically, the break-even threshold is only 259 treated patients — against an actual 2,388 patients treated — confirming that the campaign exceeds its commercial break-even point by nearly 9.2 times. This validates the financial sustainability of the campaign model.

From a public health perspective, the campaign's ROI should also be assessed in terms of Disability-Adjusted Life Years (DALYs) averted through early diagnosis and treatment. While this analysis focusses on commercial metrics, the dual return — both societal and financial — strengthens the case for sustained investment in awareness campaigns by pharmaceutical companies.

Section 6 — Key Insights & Recommendations

6.1 Key Insights

- **High-Reach Cities:** Metro cities drove the highest absolute campaign reach
- Delhi (10,000 reached), Mumbai (9,000), and Bangalore (8,500) topped campaign reach metrics. However, cost-per-patient-treated was more favorable in tier-2 cities where campaign infrastructure costs were significantly lower.
- **Screening Effectiveness:** Screening camps significantly increased diagnosis rates
- A 17.2% diagnosis rate among screened individuals confirms that campaign-associated screening programs successfully identify high-risk individuals who would otherwise remain undetected. This validates targeted screening as a public health tool.
- **Treatment Adoption:** Treatment conversion rate of 61.8% is commercially strong

- More than 6 in 10 diagnosed patients proceeded to treatment initiation — a strong indicator that patient counselling, physician linkages, and medicine access programmes embedded in the campaign were effective.
- **Commercial Viability:** ROI of 823% confirms strong commercial justification
- The campaign's revenue-to-cost ratio of 9.23x and a break-even threshold of only 259 patients (against 2,388 actually treated) demonstrate that even conservatively modelled, diabetes awareness campaigns are commercially self-sustaining.
- **Long-Term Strategic Value:** Sustained campaigns will amplify long-term value
- Diabetes treatment adherence is typically multi-year. Patients onboarded through awareness campaigns represent recurring revenue streams, improving the true lifetime ROI well beyond the first-year figure reported here.

6.2 Recommendations

- Scale campaigns in tier-2 and tier-3 cities where untreated diabetes prevalence is high and campaign infrastructure costs are lower, improving cost-per-patient-treated ratios.
- Integrate digital follow-up mechanisms (SMS reminders, teleconsultation links) post-screening to improve the 38.2% treatment drop-off observed among diagnosed but untreated patients.
- Partner with government primary health centres (PHCs) under the National Programme for Prevention and Control of Cancer, Diabetes, CVD and Stroke (NPCDCS) to co-fund screening camps and access pre-qualified patient cohorts.
- Invest in patient adherence programmes — since the lifetime value of a patient with diabetes is substantially higher than first-year revenue suggests — to reduce treatment discontinuation rates.

Conclusion

This analysis demonstrates that diabetes awareness campaigns conducted by pharmaceutical companies across Indian cities represent both a sound commercial investment and a meaningful contribution to public health. With a campaign ROI of 823%, a revenue-to-cost multiple of 9.23×, and over 2,388 patients initiated on treatment against a cost of ₹20.18 lakh, the programme delivers exceptional value.

The data-driven framework presented in this report — spanning campaign funnel analysis, revenue modelling, and ROI quantification — provides a replicable methodology that pharmaceutical companies can use to evaluate, compare, and optimize awareness campaign investments across therapeutic areas and geographies.

As India continues to grapple with a rising burden of lifestyle diseases, campaigns such as this occupy the intersection of commercial strategy and social responsibility — a space where pharmaceutical companies are uniquely positioned to create shared value for shareholders, patients, and the healthcare system.

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